

SECTION 7

The Dual Healthcare System

Two broken systems, one Indigenous patient: the Non-Insured Health Benefits program, Alberta Health Services, and what happens when 150 years of a treaty medicine chest promise becomes a 983,000-person federal program layered on top of a provincial primary care system that is collapsing in real time

Treaty 6 was signed in 1876. The medicine chest clause promised that a medicine chest would be kept at the house of each Indian agent for the use and benefit of the Indians. One hundred and fifty years later, the medicine chest has become a \$1.87 billion federal program with a three-level appeal process, routinely denied by private insurers acting as payer of first resort, declined by pharmacists unwilling to bill it, and operating alongside a provincial primary care system in which 650,000 Albertans have no family doctor at all. The promise persists. Its fulfilment does not.”

This section documents the healthcare reality facing Indigenous disabled Albertans. The structure parallels Sections 4 and 6: a jurisdictional fracture that places Indigenous health under two administratively separate systems, neither of which is adequate on its own and neither of which treats the failure of the other as its problem. The federal Non-Insured Health Benefits (NIHB) program, administered by Indigenous Services Canada and originally established as an evolution of the Treaty 6 medicine chest promise, provides supplementary coverage for prescription drugs, dental care, vision care, medical supplies, mental health counselling, and medical transportation. The provincial Alberta Health Services (AHS) system, administered under the Canada Health Act, provides physician services, hospital care, and emergency response. For a status First Nations person living off-reserve in Alberta, both systems must function in coordination. The documented record establishes that neither does.

Four findings structure the section. First, that the Treaty 6 medicine chest promise has been operationalized into NIHB in a form that the Standing Committee on Indigenous and Northern Affairs has itself identified as requiring eighteen specific reforms. Second, that NIHB functions as payer of last resort in a manner that creates administrative delay, pharmacy refusal, and out-of-pocket burden for the very population the program was established to serve. Third, that the Alberta provincial primary care system is in documented collapse, with 650,000 Albertans without a family physician, 887 accepting practices in 2020 reduced to 164 by 2024 — a collapse that affects all Albertans but compounds for Indigenous patients who must also navigate NIHB. Fourth, that anti-Indigenous racism in Canadian healthcare is not incidental but

documented across multiple public inquiries, peer-reviewed scoping reviews, and two named deaths — Brian Sinclair in 2008 and Joyce Echaquan in 2020 — whose circumstances have been formally recognized as the direct product of systemic bias in the provincial healthcare systems that deliver physician and hospital services to Indigenous patients.

Part 1: From the medicine chest to NIHB

The Treaty 6 medicine chest clause

Treaty 6 was signed in 1876 between the Crown and the Plain and Wood Cree, Assiniboine, and other peoples whose traditional territories include much of what is now central Alberta and Saskatchewan. Treaty 6 is the only numbered treaty that specifically references healthcare — the medicine chest clause, which provided that a medicine chest would be kept at the home of each Indian agent for the use and benefit of the Indians of the band. The clause was not an administrative footnote. It was a reciprocal promise offered by the Crown in exchange for the surrender of territory that, in 2026, produces the oil sands revenue and agricultural output that underwrites Alberta's provincial fiscal position.

Many First Nations groups have asserted that the medicine chest clause, taken together with the oral history of treaty negotiations and the broader fiduciary relationship between the Crown and First Nations, signals a constitutional obligation on the federal government to provide healthcare to Status First Nations persons. The legal interpretation has been contested; the political interpretation has been sustained. NIHB, administered through the First Nations and Inuit Health Branch of Indigenous Services Canada, represents the federal government's operational response to this obligation.

The scale of NIHB today

As of March 2025, NIHB provided coverage to approximately 983,000 eligible clients nationally. Federal spending on the program has averaged \$1.87 billion per year over the five years prior to the publication of this report. Coverage categories include: prescription drugs and over-the-counter medications; dental care; vision care; medical supplies and equipment (including wheelchairs, orthotics, prosthetics, hearing aids); short-term crisis intervention mental health counselling; and medical transportation to services not available on reserve or in the client's community of residence.

In Alberta and British Columbia specifically, NIHB also funds provincial health premiums for eligible clients. In British Columbia, the provincial-level coordination operates differently: the First Nations Health Authority (FNHA), established in 2013, administers federal and provincial First Nations health funding through a single Indigenous-led authority, and the FNHA has effectively replaced NIHB as the primary benefits administrator for Status First Nations persons in BC. No equivalent structure exists in Alberta. Alberta Indigenous clients continue to navigate NIHB through the standard federal administrative process.

What NIHB clients actually experience

The Standing Committee on Indigenous and Northern Affairs issued eighteen formal recommendations for NIHB program reform in 2022 — the product of hearings, testimony from healthcare professionals and Indigenous leaders, and a study of the program's documented shortcomings. These included: formal government recognition of traditional Indigenous healers; improvement of medical transportation benefit criteria; modernization of the approvals process to reduce delays; and structural reform of the program's payer-of-last-resort position. As of the publication of this report, the majority of those eighteen recommendations remain unimplemented.

The CBC Indigenous investigation published in January 2026 by journalist Joy SpearChief-Morris (2025 CJF-CBC Indigenous Journalism Fellowship recipient) documented the patient experience of navigating NIHB through the case of Patricia McCartney, a member of Six Nations of the Grand River in southern Ontario and primary caregiver for her elderly mother. McCartney reported that obtaining NIHB coverage for her mother's incontinence supplies — adult diapers, bed pads, catheters, dressing aids — required six to eight months of paperwork between the prescribing physician and the pharmacy. During that time, the mother's care needs did not pause. McCartney described the process as “so daunting” that many caregivers in her position simply give up and pay out of pocket.

Amy Lamb, Executive Director of Indigenous Pharmacy Professionals of Canada, documented the pharmacy-side reality in the same CBC reporting. Lamb stated that some pharmacies are refusing to bill NIHB at all because of the administrative burden. The program's position as payer of last resort — meaning NIHB pays only after provincial/territorial or private insurance has declined — creates documented problems in two directions. First, it produces administrative delay: when a client has any alternative coverage, that coverage must be pursued first, and only when it is declined can NIHB be billed. Second, it leads to program underfunding: because NIHB only pays for what others have declined, the actual cost of services delivered to Indigenous clients never appears fully in the NIHB budget. The Native Women's Association of Canada has documented, in its NIHB policy analysis, that Indigenous clients routinely pay out of pocket for costs not covered by NIHB, and that First Nations, Inuit, and Métis persons excluded from NIHB due to status criteria face the highest health costs of any group in Canada.

The payer-of-last-resort structure means NIHB only pays for what other insurance has already declined. Which means the actual cost of healthcare delivered to Indigenous clients never appears in the NIHB budget. Which means the program's stated expenditure of \$1.87 billion per year understates what Indigenous healthcare actually costs — because a significant portion of that cost is being absorbed by Indigenous

clients themselves, in the form of out-of-pocket payments and foregone care.”

Who NIHB excludes

NIHB eligibility is restricted to registered Status First Nations persons under the Indian Act and recognized Inuit persons. Non-status First Nations persons, Métis persons not covered by a separate Land Claim organization, and First Nations persons whose status was affected by the gender discrimination provisions of earlier versions of the Indian Act are not eligible for NIHB. As documented in Section 3 of this report, the jurisdictional fracture of Canadian Indigenous policy means that these excluded populations fall into a specific gap: they are too Indigenous to be comprehensively covered by provincial systems calibrated to the non-Indigenous population, and not Indigenous enough in the administrative sense to receive federal supplementary coverage. The result is a documented health cost differential that Native Women’s Association of Canada identifies as the highest in Canada for this specific population.

Métis and non-status First Nations persons with complex medical conditions or disabilities must navigate Alberta Health Services as their primary system of coverage, with no parallel supplementary coverage for medications, dental care, vision care, or medical supplies and equipment beyond what provincial Health Benefits Cards provide. For AISH recipients specifically, this differential is significant: the AISH Health Benefits Card covers some but not all supplementary benefit categories, and the gap for a Métis or non-status AISH recipient is documented in the Alberta Income and Employment Supports Policy Manual as explicitly different from the gap for a Status First Nations person, whose federal NIHB coverage operates alongside provincial benefits.

Part 2: The provincial side — Alberta’s primary care collapse

What 650,000 without a GP means

The Alberta Disability System Breakdown Report Series Document 5 of 12, the Healthcare Worker Shortage Report, documented the state of Alberta primary care as of April 2026. The findings, drawn from the Alberta Medical Association, University of Calgary research, ThinkHQ polling of family physicians, and peer-reviewed publication in the Canadian Family Physician journal:

- More than 650,000 Albertans have no family doctor — a population larger than the entire city of Calgary.
- The number of family practices accepting new patients fell from 887 in 2020 to 164 in 2024 — an 81 percent reduction in four years.
- 61 percent of existing family physicians are considering leaving the Alberta provincial healthcare system. Over 25 percent are considering leaving healthcare entirely.

- 4,374 family physicians were registered in Alberta as of the first quarter of 2024, 215 more than the same period the previous year — yet they are not taking new patients. The paradox is structural, not arithmetic: Alberta has more registered family physicians than ever, and fewer accepting patients than ever.
- The Canadian Family Physician journal in 2025 estimated that an additional 678 to 715 family physicians — or alternatively 722 nurse practitioners — would be required to close the Alberta primary care gap.
- Dr. Braden Manns of the University of Calgary, lead investigator on Alberta primary care research published in September 2025, stated: “People are older and sicker. The number of people with five or more health conditions — like a history of cancer, high blood pressure and heart issues — nearly doubled.”

In the Bow Valley region, the nearest GP accepting new patients was a 90-minute drive away as of early reporting. Daphne Pauls, a 53-year-old Alberta resident living with fibromyalgia whose family doctor left the province, told CBC News: “Not knowing is difficult. Consistent care is key for maintaining my health. It just leaves me kind of not knowing how will my health be cared for.”

What primary care collapse means for Indigenous patients specifically

For an Indigenous person in Alberta — Status or non-status, on-reserve or off — the provincial primary care collapse produces several specific harms. First, NIHB coverage requires a prescribing physician for most benefits. No GP means no prescriber for ongoing medications. Walk-in physicians, as documented in the Alberta Disability System Breakdown Healthcare Worker Shortage Report, routinely decline to complete complex disability forms for patients they have seen once. Second, specialist referrals require a GP. Without a GP, an Indigenous patient with a chronic condition arising from the environmental exposures documented in Sections 5 and 6 has no pathway to the specialist care that condition requires. Third, AISH, ADAP, and Disability Tax Credit applications all require physician-completed forms. An Indigenous person whose disability traces to contaminated water, inadequate housing, or multigenerational trauma documented in Section 1 cannot access the provincial disability benefit the condition qualifies them for, because the condition is real but the paperwork pathway is unavailable.

Rural reserves compound this barrier. Of the Alberta First Nations communities identified in the Indigenous Services Canada catalogue, a significant portion are located more than 90 minutes from an urban centre with primary care capacity. For an Indigenous person living on reserve and requiring off-reserve specialist care, the Section 4 housing instability, the Section 6 water contamination, and the Section 7 primary care collapse converge: the reason they need the specialist is produced by the housing and water situation; the primary care pathway to the specialist does not exist; and the transportation benefit that would bring them to the specialist is administered by NIHB's payer-of-last-resort system, which requires documentation from the physician they do not have.

Part 3: Anti-Indigenous racism as a documented feature of Canadian healthcare

Two deaths, twelve years apart, same pattern

The peer-reviewed literature on anti-Indigenous racism in Canadian healthcare, the official coroner's findings on two specific deaths, and the Turpel-Lafond 2020 "In Plain Sight" report commissioned by the Government of British Columbia establish, with primary-source documentation, that anti-Indigenous racism is a structural feature of the provincial healthcare systems that deliver physician and hospital care across Canada. The two named cases are not exceptional incidents. The peer-reviewed scoping review literature characterizes them as representative of a pattern.

Brian Sinclair, a 45-year-old First Nations man and double amputee, entered the emergency room at Winnipeg's Health Sciences Centre in September 2008 seeking care for a treatable bladder infection. He waited 34 hours. During that time, hospital staff did not ask him why he was there. Nurses did not assist him even as he vomited on himself. He was eventually found dead in his wheelchair. The inquest into his death concluded he had never been asked if he was waiting for medical care. The book *Structures of Indifference: An Indigenous Life and Death in a Canadian City*, co-authored by Mary Jane Logan McCallum and Adele Perry, analyzed the systemic factors that produced this outcome.

Joyce Echaquan, a 37-year-old Atikamekw mother of seven, entered the medical centre in Joliette, Quebec in September 2020 with stomach pains. She recorded herself on Facebook Live as nurses insulted her with racist and degrading comments while she cried out in pain. One nurse called her "stupid as hell." She died shortly afterwards. The 2021 final report from the Quebec coroner's inquest concluded that racism and prejudice from hospital staff contributed to her death. In July 2023, the Manawan Atikamekw Council, representing Joyce Echaquan's nation, opened the Joyce's Principle Office, which works to embed Joyce's Principle — an Indigenous-led framework for culturally safe healthcare — into Canadian health and social service practice.

Mary Jane Logan McCallum observed, following Joyce Echaquan's death, that the pattern was the same as Brian Sinclair's. Twelve years had passed. The public inquiries had been convened. The recommendations had been tabled. The pattern was the same.

The peer-reviewed documentation

The peer-reviewed literature on anti-Indigenous racism in Canadian healthcare is substantial and the findings are consistent. The key sources include:

- Turpel-Lafond, Mary Ellen. "In Plain Sight: Addressing Indigenous-specific Racism and Discrimination in B.C. Health Care." Government of British Columbia. 2020. The report followed allegations of a "price is right" game in which BC emergency department staff guessed Indigenous patients' blood alcohol levels. The resulting provincial review documented

widespread anti-Indigenous racism in the BC health system at systemic, institutional, and interpersonal levels.

- Browne, Annette J., Josée Lavoie, Mary Jane Logan McCallum, and Christa Big Canoe. “Addressing anti-Indigenous racism in Canadian health systems: multi-tiered approaches are required.” *Canadian Journal of Public Health*. 2022. Peer-reviewed commentary calling for anti-racism to be adopted as a sixth pillar of the Canada Health Act, on the grounds that structural change is required and one-off responses to specific racist incidents are insufficient.
- Cooke, Martin, and Tyler Shields. “Anti-Indigenous racism in Canadian healthcare: a scoping review of the literature.” *International Journal for Quality in Health Care*. 2024. Peer-reviewed scoping review establishing that the majority of racism experiences documented in the literature are “covert” rather than overt — meaning they are difficult for the Indigenous patient to identify with certainty in the moment, even when their effects on health outcomes are real and measurable.
- Adegoke, Kola, Abimbola Adegoke, Deborah Dawodu, and Temitope Kayode. “Systemic Racism in Canadian Healthcare: A Policy and Equity Analysis.” *Journal of Primary Care & Community Health*. 2025. Qualitative analysis of public inquiries, government audits, and case files across four central areas: institutional discrimination, licensing and workforce exclusion, patient and cultural safety, and accountability gaps. The analysis finds continuing institutional disregard for Indigenous patients, most marked in emergency and maternal services.

The Chadha 2020 commentary, cited repeatedly in this literature, frames the specific intersection this section addresses: the combination of Indigenous identity and disability produces compounded harm in healthcare encounters. Chadha’s commentary is titled “Why it’s dangerous to be disabled and Indigenous in Canada.” That is not a rhetorical claim. It is the characterization of a documented pattern drawn from primary evidence.

The peer-reviewed literature names it directly. It is dangerous to be disabled and Indigenous in Canada. Not because the disability is dangerous. Because the healthcare system, as documented across multiple provinces, multiple inquiries, and multiple decades, produces harm to disabled Indigenous patients through structural and interpersonal bias that has been studied, named, and insufficiently changed.”

Part 4: The intersection for Indigenous disabled Albertans

What it takes to navigate both systems simultaneously

An Indigenous disabled Albertan — the specific population this report documents — must navigate both systems simultaneously. The following is what that navigation involves on a

typical month, assuming moderate complexity of medical need (multiple chronic conditions, at least one specialist, prescription medications, and some category of medical supply):

- First: locate a family physician. In a province with 650,000 without a GP and 164 accepting practices as of 2024, this may require months of active searching or may be simply unavailable in the patient's community of residence.
- Second: if the physician is found, schedule a visit. For a Status First Nations person, the visit is billed to the provincial AHS system; this is not a NIHB transaction. If prescription medications are prescribed, the physician's prescription goes to a pharmacy that must attempt to bill NIHB — after first attempting to bill any other coverage the patient may have.
- Third: if the patient requires specialist care, the GP must initiate a referral. Specialist wait times in Alberta are routinely six to eighteen months for non-urgent specialties. During that wait, the underlying condition continues.
- Fourth: if the patient requires medical transportation to a specialist not available in their community, a separate NIHB medical transportation request must be submitted, approved under the NIHB Medical Transportation Policy Framework, and coordinated through an NIHB regional office or a First Nations/Inuit health authority. Denial of medical transportation is appealable through a three-level process. The appeal must be submitted in writing.
- Fifth: if the patient requires medical supplies or equipment — a wheelchair, orthotics, a hearing aid — a separate NIHB request is submitted, subject to prior approval, and bill-able to NIHB only after other coverage has declined.
- Sixth: if the patient is an AISH or ADAP recipient, a physician-completed medical report is required for provincial disability benefit eligibility — an additional form the physician must complete, which may or may not be billed to the patient personally as a fee-for-service charge under Alberta's primary care reform framework.
- Seventh: if the patient encounters racism or inadequate care at any stage in the encounter, the formal complaint mechanisms available are the College of Physicians and Surgeons of Alberta, the Alberta Human Rights Commission, the Office of the NIHB Regional Medical Officer, or — for BC residents under the FNHA — the In Plain Sight implementation framework. None of these mechanisms is designed for rapid response. None of them prevents the harm that has already occurred.

Each of these seven steps functions in isolation. None of them coordinates with the others. A missed step at any stage can render the subsequent steps inaccessible. For a patient with the cognitive, communication, or literacy profile that defines a significant portion of the AISH-qualifying population, the cumulative navigation burden is structurally prohibitive.

The ADAP transition and the Indigenous patient

The July 1, 2026 ADAP transition adds a further dimension. As documented throughout this report series, ADAP requires all 79,290 current AISH recipients to undergo reassessment and, if remaining on AISH, to obtain a physician-completed Disability Assistance Medical Report. For an Indigenous patient without a family physician, operating within the NIHB payer-of-last-resort framework, the physician-completed form requirement is not a procedural step. It is a structural barrier that, in combination with the primary care collapse documented in Part 2 and the racism documented in Part 3, may prevent continuation of the disability benefit that the patient's condition qualifies them for under the program's own written criteria.

The Indigenous patient whose disability traces to the causes documented in Sections 1, 4, 5, and 6 of this report — residential schools, inadequate housing, contaminated land, poisoned water — faces a cascade: the disability is real, the causal pathway is documented, the federal program that ought to cover supplementary needs (NIHB) operates as payer of last resort and is declined by pharmacies and denied in the McCartney pattern, the provincial primary care system that ought to document the condition is collapsed at 164 accepting practices, the provincial disability benefit that ought to support the recipient is being transitioned under ADAP with a physician-completed form requirement the patient cannot fulfill, and the entire cascade is administered by two levels of government neither of which treats the failure of the other as its responsibility.

What this section establishes for the remainder of the report

The Treaty 6 medicine chest promise has been operationalized into NIHB, a federal program now serving approximately 983,000 Status First Nations and Inuit clients at an annual cost of \$1.87 billion. Eighteen formal reform recommendations from the Standing Committee on Indigenous and Northern Affairs remain largely unimplemented. NIHB functions as payer of last resort, producing documented administrative delay, pharmacy refusal, and out-of-pocket burden. The provincial Alberta Health Services system is in documented primary care collapse, with 650,000 Albertans without a GP and the number of accepting practices reduced 81 percent in four years. Anti-Indigenous racism in the provincial healthcare systems that deliver physician and hospital care is peer-reviewed, inquiry-documented, coroner-confirmed, and named in the specific cases of Brian Sinclair (2008) and Joyce Echaquan (2020) as contributing to their deaths. For Indigenous disabled Albertans navigating both systems simultaneously, the cumulative structural burden is prohibitive.

The pattern across Sections 4, 5, 6, and 7 is now consistent. Housing that federal funding has failed to make adequate. Land that federal and provincial regulatory inaction has allowed to be contaminated. Water that federal advisories and provincial resource extraction have rendered unsafe. Healthcare that dual federal-provincial administration delivers to Indigenous patients with documented racial bias and structural barrier. Each of these four sections describes a separate dimension of the conditions under which Indigenous disabled Albertans live. None of them describes an isolated problem. All of them describe the operating environment of the same

population, a population whose life expectancy — as documented in Section 2 — is 19 years below the non-First Nations Alberta average.

Sections 8 through 15 will address the remaining dimensions: Jordan's Principle and children's services, child welfare, the justice system, MMIWG2S+, consultation failure and UNDRIP Article 21, the Alberta Sovereignty Act and its relationship to Treaty, TRC Calls to Action 18 through 24, and a synthesis of the July 1, 2026 ADAP transition in its Indigenous dimension. The population the report documents is the same throughout. The structural pattern the report documents is the same throughout. The remedy this report calls for is the same throughout: an honest federal and provincial acknowledgment that the outcomes documented here are not administrative accidents but the cumulative product of policy choices that can be made differently.

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